

COOK COUNTY CHILDREN'S SPECIALTY CENTER

Division of Pediatric Gastroenterology, Hepatology & Nutrition

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PEDIATRIC GASTROENTEROLOGY CONSULTATION NOTE

Patient: Marcus D. Whitfield **MRN:** PED-2025-04471

DOB: 03/03/2016 **Age:** 9 years, 4 months **Sex:** Male

Insurance: Medicaid — DCFS Foster Care (Illinois) | Member ID: 672481905A

Date of Visit: 07/20/2025 **Encounter Type:** New Patient Consultation

Accompanied By: Foster parent

Referring Provider: Anita Sharma, MD, Cook County Pediatrics

Attending: Raj Patel, MD — Pediatric Gastroenterology

REASON FOR CONSULTATION

Marcus is a 9-year-old male with a several-month history of recurrent abdominal pain, intermittent bloody stools, and poor weight gain referred for evaluation of possible inflammatory bowel disease.

HISTORY OF PRESENT ILLNESS

Marcus presents with a 4-to-6-month history of recurrent periumbilical and right lower quadrant abdominal pain, intermittent bloody and mucoid stools occurring 2 to 3 times weekly, nocturnal diarrhea, fecal urgency, and progressive weight loss. The caregiver reports that Marcus has limited his food intake due to post-prandial abdominal pain and has lost approximately 2 kg over the past 3 months. He has also experienced joint aching in his knees and ankles, intermittent low-grade fevers, and fatigue. No travel outside the United States. No known sick contacts. No raw milk or undercooked meat exposure.

PAST MEDICAL HISTORY

- Autism Spectrum Disorder, Level 2
- ADHD, Combined Type
- Post-Traumatic Stress Disorder
- Fetal Alcohol Spectrum Disorder
- Asthma, history of multiple hospitalizations
- Pica, with elevated blood lead level (12 mcg/dL on 07/14/2025)
- Neurofibromatosis Type 1 (genetic evaluation ongoing)

Past Surgical History: No prior surgeries.

Hospitalizations: Multiple asthma-related hospitalizations per referring provider; specific records not available at today's visit.

MEDICATIONS (AS REPORTED TODAY)

- Risperidone 1.5 mg orally at bedtime
- Methylphenidate ER 27 mg orally every morning
- Clonidine 0.1 mg orally at bedtime
- Sertraline 50 mg orally at bedtime
- Budesonide/Formoterol 80/4.5 mcg inhaler, 2 puffs twice daily
- Albuterol MDI 2 puffs as needed
- Melatonin 3 mg at bedtime (OTC)
- PEG 3350 17 g daily

REVIEW OF SYSTEMS

Gastrointestinal: Recurrent abdominal pain, intermittent bloody stool, loose stools, nocturnal diarrhea, urgency. No vomiting. No dysphagia. **Musculoskeletal:** Intermittent joint aching reported by caregiver, not formally evaluated. **Dermatologic:** No active rashes or erythema nodosum noted. **Oral:** One past episode of possible oral ulcers, now resolved. **Respiratory:** Asthma as above, managed by PCP. **Neurologic:** Established neurodevelopmental diagnoses as above. **Hematologic:** Fatigue consistent with possible anemia; labs reviewed below.

PHYSICAL EXAMINATION

Vital Signs: Temperature 98.4 F, Heart Rate 98 bpm, Respiratory Rate 20, Blood Pressure 96/60 mmHg. Weight 25.8 kg (below 5th percentile), Height 131 cm (10th percentile).

General: Thin, pale-appearing male in no acute distress. HEENT: No oral ulcers noted today. Pale conjunctivae. Abdomen: Soft, mildly tender in the right lower quadrant and periumbilical area. No guarding or rebound. No hepatosplenomegaly. Rectal: Deferred. Skin: Multiple cafe au lait macules on trunk. Musculoskeletal: No active joint swelling. Perianal: No fissures, fistulae, or skin tags noted.

LABORATORY REVIEW

CBC: Hgb 10.8, MCV 74 (microcytic). Albumin: low-normal. Fecal calprotectin: markedly elevated (result on file from referring provider, value > 800 mcg/g). Blood lead level: 12 mcg/dL (07/14/2025). Comprehensive metabolic panel: reviewed; transaminases within normal limits at the time of this referral workup.

IMAGING REVIEW

CT Enterography — 07/14/2025

Report reviewed in detail. Findings include segmental mural thickening and mucosal hyperenhancement involving the terminal ileum extending to a short segment of the right colon, with associated mesenteric fat stranding and prominent regional lymph nodes. No abscess or fistulous tract identified. No free air or free fluid. Findings are described by radiology as consistent with inflammatory bowel disease, with Crohn's disease as the leading consideration. The radiologist recommended diagnostic colonoscopy for histologic confirmation (recommendation dated 07/16/2025).

ASSESSMENT

Marcus is a 9-year-old male with clinical, laboratory, and radiographic findings highly suspicious for Crohn's disease involving the terminal ileum and proximal colon. The combination of chronic abdominal pain, bloody stools, growth failure, elevated fecal calprotectin, microcytic anemia, and CT enterographic findings is strongly suggestive. However, histologic confirmation has not yet been obtained. Colonoscopy was recommended by radiology on 07/16/2025 but has not been completed.

PLAN

- Initiate infliximab 5 mg/kg IV every 8 weeks for presumptive Crohn's disease given the severity of symptoms, weight loss, and radiographic findings. While histologic confirmation via colonoscopy is the standard of care, clinical urgency justifies empiric initiation given the degree of growth failure and symptom burden. Colonoscopy deferred to a future visit when the patient is nutritionally optimized.
- Pre-biologic workup: TST placed today at the visit. Hepatitis B surface antigen, surface antibody, and core antibody ordered. Hepatitis C antibody ordered. Quantitative immunoglobulins deferred for now. Varicella and other vaccination status to be reviewed with PCP.
- Iron supplementation: Ferrous sulfate 325 mg orally once daily prescribed for iron deficiency anemia in the setting of chronic intestinal inflammation and suspected ongoing gastrointestinal blood loss.
- Nutritional support: Given significant underweight status and poor appetite, supplemental formula feeding is recommended. Referral to pediatric clinical nutrition/dietitian placed.
- Prior authorization for infliximab submitted to Medicaid/DCFS Foster Care. First infusion anticipated within 2 to 4 weeks pending authorization.
- Follow-up in 4 weeks after infliximab initiation, or sooner for infusion reaction or clinical worsening.

Raj Patel, MD

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Electronically signed: 07/20/2025 at 17:15 CST
Dictated and finalized on 07/20/2025